

Derbyshire Shared Care Pathology Guidelines

UHDB Paediatric Allergy service

Purpose of Guideline

- Guidance on how to identify those children who have allergy or suspected allergy
- Outline the process and indication for referral to paediatric allergy clinic

Background

Food allergy is an adverse immune response to a food. It is extremely common with at least 6-8% of the childhood population suffering from allergy to one or more foods. It can be classified into IgE-mediated and non-IgE-mediated reactions. Some reactions involve a mixture of both IgE and non-IgE responses and are classified as mixed IgE and non-IgE allergic reactions.

Food allergy may be confused with food intolerance, which is a non immunological reaction that can be caused by enzyme deficiencies, pharmacological agents and naturally occurring substances. Food intolerance will not be covered in this guideline. The starting point for the guideline is a suspicion of allergy and the use of an allergy-focused clinical history to help determine whether an allergy is likely.

It is important to note that up to one third of all parents report allergy to foods in their children but the true incidence of food allergy is much lower (6-8%). In those parents who cannot be reassured referral may be necessary to offer a 'second opinion'.

Obtaining a useful clinical history

Please ask for occurrence of all of these symptoms:

IgE mediated (Acute onset)	Non IgE mediated (Delayed onset)
Skin • Pruritis • Erythema • Acute urticaria (localised/generalised) • Acute angioedema (face, eyes, lips) Gastrointestinal • Angioedema of lips/tongue/palate • Oral pruritis • Colicky abdominal pain • Vomiting and diarrhoea Respiratory • Upper respiratory (nasal itching, sneezing, rhinorrhoea, congestion) • Lower respiratory (cough, wheeze, chest tightness, shortness of breath)	Skin • Pruritis and erythema • Atopic eczema Gastrointestinal • Gastrooesophageal reflux disease • Loose/frequent stools • Blood and/or mucus in stools • Abdominal pain • Infantile colic • Constipation • Food refusal or aversion • Peri-anal redness • Pallor/ tiredness • Faltering growth (with one or more of above)

The timings of any reactions or symptoms need to be recorded in relation to possible allergen exposure. The following should also be identified:

Identify allergen Food, insect/sting, animal, pollen, medication
 ?First exposure, ?previous exposure
 Known allergen, other allergies

Identify any other triggers/ co-factors - Temperature, exercise induced, inter-current illness

Identify atopic tendency – Asthma, eczema, hay fever

Identify medications – Antihistamine, steroids, beta 2 antagonists, inhaled medication

Identify strong family history – Parents, first degree relatives with asthma, hay fever, food allergies, other known allergies

Investigations

Children should not be subjected to Specific IgE testing looking for a possible cause of their symptoms, if it is not easily identifiable on history. Children can be referred to the paediatric allergy clinic for Skin Prick Tests +/- Specific IgE tests if this is required to help make a diagnosis.

Paediatric Allergy Service

The service offers a one stop shop approach to managing paediatric patients with allergy offering: Skin prick testing, specific IgE / blood test workups, personalised allergy management plans, prescription and training of adrenaline auto injector pens.

The service sees children who have food allergies but also sees and treats children with difficult to manage hay fever, allergic rhinitis, atopic eczema and asthma. The service offers desensitisation programmes to cows milk and also to grasses, pollens and HDM; although these are to a very specific group of patients who have not responded to maximal medical therapies. Please see separate referral guidelines for these patients.

There are also individual primary care guidelines for specific management of:

- Paediatric Egg Allergy - please see separate "Management of children with egg allergy and referral to paediatric allergy clinic" guideline on the Shared Care Pathology website
- 'Infants under one year with cows milk protein allergy'
http://www.derbyshiremedicinesmanagement.nhs.uk/assets/Clinical_Guidelines/Formula_by_BNF_chapter_prescribing_guidelines/Appendix_5/Infant_feeding_guideline.pdf

Criteria for referral to paediatrics

Based on the allergy-focused clinical history, NICE recommends that referral to secondary or specialist care should be considered in any of the following circumstances in paediatric patients:

The child or young person has:

- Faltering growth in combination with one or more of the gastrointestinal symptoms described in the table above (see page 1)
- Not responded to a single-allergen elimination diet
- Had one or more acute systemic reactions
- Had one or more severe delayed reactions
- Confirmed IgE-mediated food allergy and concurrent asthma
- Significant atopic eczema where multiple or cross-reactive food allergies are suspected by the parent or carer. Locally these need to have been reviewed by dermatology to ensure that eczema management is optimised before referral. Dermatology will refer to Allergy clinic if required.

OR there is:

- Persisting parental suspicion of food allergy (especially in children or young people with difficult or perplexing symptoms) despite a lack of supporting history
- Strong clinical suspicion of IgE-mediated food allergy but allergy test results are negative or the allergen is not easily identified
- Clinical suspicion of multiple food allergies

Useful Contacts

Appointments are available via Choose and book. Three consultants carry out paediatric allergy clinics in Derby and referrals may be discussed prior to booking if there are any queries:

Dr Donna Traves (Consultant Paediatrician)
Telephone 01332 786441
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Dr Lizzie Starkey (Consultant Paediatrician)
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Dr Shilpee Sinha (Consultant Paediatrician)
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References

1. Food allergy in under 19s: assessment and diagnosis. NICE CG116. Issued February 2011. <https://www.nice.org.uk/guidance/cg116>
2. Anaphylaxis: assessment and referral after emergency treatment NICE CG134. Updated Aug 2020. <https://www.nice.org.uk/guidance/cg134>
3. EAACI Food Allergy and Anaphylaxis Guidelines: diagnosis and management of food allergy. Allergy 2014 Aug;69(8):1008-25. doi: 10.1111/all.12429. Epub 2014 Jun 9 <https://pubmed.ncbi.nlm.nih.gov/24909706/>

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